

01

LECTURE

HEALTH CARE

MT 1, Second Semester LECTURE 01 - Mr. Michael David C. Gu, RMT



FOUNDATIONS OF PUBLIC HEALTH

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LEARNING OBJECTIVES

- Describe the historical development of public health practices.
- Identify key milestones in public health history.
- Understand the evolution of public health concepts.
- Recognize major public health challenges in the 21st century.
- Differentiate levels of healthcare and their applications.
- Highlight the role of community participation in improving health outcomes

I. HISTORY OF PUBLIC HEALTH

A. HISTORICAL ROOTS

Indigenous & Tribal Societies

- Believed that diseases were caused by **malevolent spirits or bad luck**
- Presence of **shamans and witch doctors** who were believed to:
 - be able to communicate with the spirit world
 - Conduct rituals to drive away evil spirits and restore health
- Usage of **herbs and plants as medicine**

Babylonians or Mesopotamians

- One of the first civilizations
- Early understanding of the **importance hygiene**
- Developed basic **medical skills**, which set the foundation for organized health practice

Ancient Egyptians

- Physicians or doctors were mostly **priests**
 - Medicine and religion were intertwined
- Developed a range of pharmaceutical preparations
- **Developed surgical skills**
 - Invented devices that appeared to be prototypes of modern day surgical instruments
 - Before bodies are mummified, organs are removed, which is a form of primitive surgery
- Prioritized **sanitation**
 - Constructed earth **privies** (first version of toilets) and **public drainage systems**
- Established rudimentary baths and toilets in dwelling places

Chinese

- **Acupuncture Therapy**
 - Believed to restore balance within the body

Hebrews | Hebrew Mosaic Law

- Stressed **prevention of disease**
 - Regulation of personal and community hygiene, reproductive and maternal health
- Isolation of lepers and other "unclean conditions"
 - To prevent spread of disease

- Incorporated guidelines on family and personal sexual conduct was part of religious practice
 - Further highlighted their commitment to health as part of spiritual wellbeing.
- Practiced quarantine

Greeks

- Pioneers to linking **health to environment**
- Laid the foundation for preventive measures
 - Wealthy people valued personal cleanliness, exercise, diet, and sanitation to prevent diseases
- Came up with the concept of the **"Four Humors"**
 - Phlegm, Blood, Yellow Bile, and Black Bile
- First to observe the human body and the effects of disease systematically
 - Led to the development of modern medical science
- Employed various therapies
 - Massage, Art Therapy, Herbal Treatments
- Up until this point, the practices when it came to healthcare were largely rooted in superstition and religious beliefs
 - Treatments were done through rituals and spiritual interventions, until the Greeks
- **Hippocrates**
 - Greek physician of the Age of Pericles
 - Considered one of the most outstanding figures in the history of medicine
 - Ushered a monumental shift on how society will approach healthcare
 - Often referred to as **the father of modern medicine**

- Noted on the effect of food, occupation, and especially climate in causing disease
 - Emphasized the role of environmental factors in health
- His works served as a practical guide, influencing decisions on how cities were selected back then in the Greco-Roman world
 - First **science-based approach** to public health
- Founded the **Hippocratic School of Medicine**, which was the first to use the terms *acute* (short term), *chronic* (long term), *endemic*, *epidemic*, and *exacerbation*

Romans

- First to provide organized medical care, primarily for their legions
- **Prevention over cure**
- Viewed medicine from a community health and social medicine perspective
- Emphasized community well-being and public welfare
- Emphasized **regulation of medical practice**
 - Ensured standards were maintained by healthcare providers
- Had a provision of purified water
- Established **sewers and aqueducts**, many of which still exist today
 - Marsh drainages reduced malaria cases
- Had supervision of public food preparation
- **Galen**
 - Greek physician who migrated to Rome
 - His works became the foundation for the study of Human Anatomy
 - His theories in the matter, dominated European medicine for the 1500 years

Middle Ages or Medieval Period | 5th - 15th Century

- Period after the fall of the Roman Civilization
- Western Europe experience a period of social and political disintegration
 - Large cities disappeared, replaced by small villages surrounding the castles of Landlords (**Feudalism**)
- Only unifying force was **Christianity**
 - Learning and culture of the Greco-Roman world was preserved in monasteries
- The discoveries, learning, practices of the Greeks and Romans were carefully preserved
 - Served as the bridge between ancient time and the medieval time
 - Because of this, healthcare practices did not need to start from the very beginning
- The age is marked by **poor sanitation**
 - Contributed to the **rise of communicable diseases**, such as cholera, smallpox, and the Bubonic Plague.
- **Bubonic Plague/The Black Plague**
 - Most devastating epidemic
 - Wiped out ⅓ of Europe and Asia
 - Around 75-200 million deaths
- Despite the challenges, there was a movement of health education and promotion of health and personal hygiene
- **Early Christians** preserved the Roman and Greek medical ideologies
 - The Roman Catholic Church playing a significant role in influencing how people looked at health and illness
 - The church's reputation was cemented because they were the ones taking care of the people

- Religious convents and monasteries established hospices to shelter travelers and sick persons
 - The caregivers are often members of religious orders
- The need for medical care, given by the Catholic Church brought into prominence the profession of **Nursing**
 - At this time, nursing as a profession wasn't clearly established, but the beginning started here
- Leprosy**
 - Most critical concern of the time
 - To combat its spread, the **practice of quarantine** or isolation was introduced, just like the Hebrews
 - Separated those affected with leprosy
 - This practice is still relevant today
 - Leper houses** (leprosia) were made to tend to those afflicted
 - Represents the earliest application of a public health intervention on a governmental level
 - The authorities of time were the ones initiating the practices
- Responsibility for communal functions transferred from **Feudal Lords and Church Leaders** to **Lay Councils** presided over by a hierarchy of hereditary or appointed officials
 - Public health activities** (e.g. overseeing the water supply and sewerage, street cleaning, supervision of markets) **fell under the jurisdiction of councils**
 - The transfer of authority marked the pivotal step to the **formation of a formal organization of public health**

Salerno Medical School | *Schola Medicina Salernitana*

- World's **first formal medical school**
- Lay organization independent of the church
 - Marked the departure of church control during that time
- Notable for welcoming students of any race or creed
- Established the first health guide for the masses, **The Salernitan Rule of Health**
 - Emphasized hygiene, diet, exercise, and temperance in health
- The school also had a diverse faculty
 - Women were part of the faculty despite being a men-centered society
 - Focused on obstetric issues
- Constantine the African (1020-1087)**
 - Peripatetic scholar
 - Follower of Aristotle and his system of philosophy
 - Made a significant contribution by translating essential Arabic works to Latin
- Al-Razi**
 - Father of Pediatrics**
 - Wrote **"The Diseases of Children"**
 - Earliest works that addresses childhood illnesses and treatments
- Avicenna**
 - Author of the **"The Canon of Medicine"**
 - Served as a major reference book for medical schools worldwide until the middle of the 16th century

Renaissance Period | *Rise of the Scholars*

- Marked a turning point in medicine
- The rise of scholars revolutionized our understanding of the human body and disease
- Girolamo Francastoro**
 - Expanded knowledge on how epidemics and infections are spread
- Andreas Vesalius**
 - Authored the **"Structure of the Human Body"**, a foundational work in human anatomy
- William Harvey**
 - Conducted groundbreaking studies on the human circulatory system and the properties of blood
- Antonie van Leeuwenhoek**
 - Used a crude microscope to discover bacteria microorganisms, starting new ideas for the cause of disease

Colonial Period

- Time of **significant disease exchange** between the colonizers and the colonized, profoundly impacting global health
 - E.g. The Galleon Trade did not just exchange supplies, but also diseases, such as parasites
- Saw significant advances in medicine, as the physicians at that time introduced remedies for the treatment of diseases
- Charles Laveran**
 - French surgeon who discovered parasites in malaria patients
 - Revolutionized our understanding of disease
- Ronald Ross**
 - Discovered that malarial parasites were transmitted by mosquitoes through mosquito bites

- Was a breakthrough in disease prevention, because knowing the cause can help find the cure

• Edward Jenner

- **Father of Immunology**
- Pioneered vaccination or immunization
- Developed the vaccine against smallpox, which was a devastating disease at that time

Industrial Revolution

- Occurred alongside the Colonial Period was the **rise of industries**
- Brought remarkable advances in transportation, communication, and other forms of technology
- This era had **significant public health challenges**
 - Rapid growth of industries led to **overcrowding of cities**
 - Overcrowding led to **poor sanitation**
- In response, religious women of that time began providing essential nursing care in institutions and homes, paving the way for modern nursing practice
 - Practice of nursing was formally established

• Edwin Chadwick

- Studied the prevalence of preventable diseases, focusing on the working poor
- Advocated for sanitation reforms

• Dr. John Snow

- **Father of Epidemiology**
- Traced the cholera outbreak to a contaminated water pump, which demonstrated how diseases spread

• Robert Koch

- Pioneered the **field of bacteriology**
- Identified specific microorganisms for diseases like Tuberculosis and Anthrax
- Koch's principle

• Louis Pasteur

- Introduced the concept of fermentation
- Developed the rabies vaccine
- Developed **pasteurization** to kill bacteria in milk, revolutionizing public health.

B. MILESTONES IN THE HISTORY OF PUBLIC HEALTH

MILESTONES IN THE HISTORY OF PUBLIC HEALTH (Note: The ones in BOLD are ones Sir Gu has said to take note of)	
1601	Elizabethan Poor Law was written
1789	Baltimore Health Department was established
1798	Marine Hospital Service Department established in Dublin where nuns visited the poor
1813	Ladies Benevolent Society of Charleston, South Carolina was founded
1836	Lutheran deaconesses provided home visits in Germany
1855	Tuberculosis campaigns in the U.S. begins
1864	Beginning of Red Cross
1928	Alexander Fleming discovered Penicillin
1946	Centers for Disease Prevention (CDC) was established

1948	After World War II, World Health Organization was established
1950s	Jonas Salk (1953) and Albert Sabin (1956) developed vaccines against Polio
1976	Peter Piot discovered Ebola Virus in Zaire, Africa
1977	Smallpox was eventually eradicated
1982	Selected Primary Health Care was established in Italy
1998	Outbreak of Swine Flu H1N1 in the U.S.
1997-2002	New strains of Swine Flu such as H3N2 and H1N2
1997	Another strain of Avian Flu virus (H5N1) hit poultry workers in Hong Kong

Table 1. Milestones in the History of Public Health

Early 20th Century

- Focused more on **infant and child mortality**
- Maternal and child health programs were initiated with emphasis on nutrition, medical care, and health inspections in schools
- High rates of **occupational diseases and industrial injuries** led to programs for industrial hygiene and occupational health

Late 20th Century

- With the decline of infant and child mortality rates, there was a focus on **identifying risk factors** for diseases of the **aging population**, such as **Diabetes and Hypertension**
- After World War II, a prominent role for behavioral factors was quickly established (diet, exercise)

20th Century

- Rise of many developments in public health associated with **social reforms**
- Saw growth of concern on diseases such as **Polio & Yellow Fever**
- Development of **retroviral treatment** that reduced the risk of death and complications due to AIDS

21st Century

- A focus on diseases such as **SARS, Ebola**, and even **COVID-19**
- **2003**: WHO discovers virus causing outbreak of SARS (Severe Acute Respiratory Syndrome)
- **2014**: there was a sudden outbreak of Ebola virus in West Africa including the U.S.A.

C. MAJOR ACHIEVEMENTS OF PUBLIC HEALTH IN THE 20TH CENTURY

- Vaccination to reduce epidemic diseases
 - e.g. of epidemic is COVID-19
- Set rules for improved motor vehicle safety
- Safer workplace safety regulations
 - To reduce the prevalence of occupational diseases
- Control of infectious diseases
- Decline in death from cardiovascular disease
 - Medicines have been made to regulate blood pressure
- Improvements in maternal and child health
- Family planning
- Fluoridation of drinking water
 - Helps with dental health
 - Strengthen teeth enamel (practiced in US)
- Reductions in prevalence of tobacco use

D. CONCEPTUAL EVOLUTION OF PUBLIC HEALTH

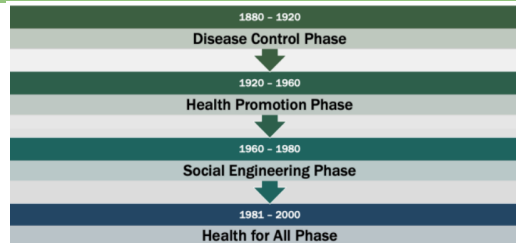


Figure 1. Conceptual Evolution of Public Health

Disease Control Phase | 1880-1920

- Focused on disease control through **sanitary regulations and vaccinations**
- To contain diseases such as malaria, which was prevalent during that time
- Focused on controlling the **physical environment**
 - Improving the water system
 - Ensuring proper sewage disposal
 - Enforcing sanitation to prevent the spread of diseases

Health Promotion Phase | 1920-1960

- Emphasized on **preventive care and healthier lifestyles**
- Shifted the focus the **individual** instead of the environment
- Programs were aimed to helping the wellbeing of the people directly
 - Mother and child health services
 - School health services
 - Industrial health services
 - Mental health and rehabilitation services

Social Engineering Phase | 1960-1980

- Focused on **social determinants of health and equity**
 - To improve infrastructure and policies
- The pattern of focus began to shift because of the advancement of preventative medicine and practice of public health
- A lot of diseases could now be controlled, however, there was a **rise of chronic diseases** such as cancer, diabetes, and cardiovascular diseases
- The concept of **risk factors** began to emerge
- Consequences of these diseases was about to place a chronic burden on the society

Health For All Phase | 1980-2000

- Focusing on **universal healthcare coverage** and a bare minimum of **primary health care for everyone**
- As centuries unfolded, the **contrast of health in developed and developing countries came into sharper focus**, despite advances in medicine.
 - Most people in developed countries enjoy all the determinants of good health such as adequate income, nutrition, education, sanitation, safe drinking water, and comprehensive health care. However, only 10-20% of the population in developing countries enjoy ready access to health services of any kind.
- Because of this, the focus was once again shifted to **include those who are not financially capable to have access to healthcare**
- In 1981, members of WHO pledged themselves to provide health for all by the year 2000.
- Still underway at this point

E. FUTURE OF PUBLIC HEALTH

- Today, it addresses a wide range of issues:
 - All infectious diseases (HIV/AIDS)
 - All chronic diseases
 - Violence
 - Injury prevention
 - Birth defects
 - Bioterrorism
- Public health continues to evolve as it responds to the dynamical changes in society, both at the local and global levels.

F. DEFINITION OF PUBLIC HEALTH

Winslow (1920)

- “The science and art of **preventing disease, prolonging life, and promoting physical health and efficiency** through organized community efforts for the sanitation of the environment, the control of community infections, the education of the individual in principles of personal hygiene, the organization of medical and nursing service for the early diagnosis and preventative treatment of disease, and the development of the social machinery which will ensure to every individual in the community a standard of living adequate for the maintenance of health.”

II. THE HEALTH FIELD CONCEPT

Four Principal Determinants of Health

- **Human Biology:** Genetic and physiological factors influencing health
- **Environment:** External factors like air, water, and living conditions
- **Lifestyle:** Personal behaviors and habits impacting well-being

- Are they a drinker? Are they a smoker?

- **Healthcare Organization:** Access to and quality of health services
 - Consists of the quantity, quality, arrangement, nature, and relationships of people and resources in the provision of health care
 - More prioritized than the other determinants
- Usually used by doctors when they do diagnosis; they ask questions based on the 4 determinants
- Further divided into dietary habits and sexual practices

Characteristics

- **Equal Importance:** To raise human biology, environment, and lifestyle to a level of categorical importance equal to that of health care organization
- **Comprehensive Framework:** Any health problem can be traced to one, or a combination of the four elements
- **Systematic Analysis:** Permits a system of analysis by which any question can be examined under the four elements in order to assess their relative significance and interaction
- **Subdivision of Factors:** Permits a further exploration of each element for a better understanding
- **New Perspectives:** Provides a new perspective on health, a perspective which frees creative minds for the recognition and exploration of hitherto neglected fields

A. PUBLIC HEALTH

Hanlon

- “Public health is dedicated to the common attainment of the highest levels of physical, mental, and social well-being and longevity consistent with available knowledge and resources at a given time and place”
- Emphasizes the **comprehensive nature of public health**
 - Doesn't focus just on the physical health, but also the mental and social well-being
- Highlights the importance of using whatever is available, whether is knowledge or resources to effectively promote equity and address the needs of the community
- The ultimate goal is to create environments where individuals and populations can thrive and enjoy long, healthy lives.

Public Health

- Primary Goal: **Prevention of disease and disability**
- It is a specialty area that is **both community-oriented and population-focused**, aiming to improve health outcomes for entire populations.
- The overall mission of public health is to **organize community efforts** that apply scientific and technical knowledge to prevent disease and promote health.
- To achieve this mission, public health operates through its three core functions:
 - **Assessment**
 - **Policy Development**
 - **Assurance**

Assessment

- **Systematic data collection on the population.** Monitoring the population's health status and making information available about health of a community
- Factors cited from WHO Community Health Needs Assessment (2001) published by the WHO Regional Office for Europe:
 - **Physical Environment** in which people live, such as the quality of the air they breathe and the water they drink
 - **Social Environment:** The level of social and emotional support people received from friends and/or family
 - **Poverty**, which shortens and reduces enjoyment of life and shortens life expectancy, making it a critical factor to address in health improvement.
 - **Behavior and Lifestyle**
 - Habits such as smoking can lead to serious health conditions like lung cancer & heart disease. Promoting healthy behaviors is essential.
 - e.g. smoking causes lung cancer and coronary heart disease. Alcohol consumption, diet, exercise, and substance abuse
 - **Family Genetics and Individual Biology** :A person's family health history and biological makeup affect their likelihood of staying healthy.
- By analyzing these factors, we can better understand a community's needs and develop effective strategies to improve health outcomes.

Policy Development

- Involves the creation of policies that support the health and well-being of the population.
- These policies are based on scientific knowledge and evidence, ensuring that decisions are informed and effective in addressing a health challenge.

Assurance

- focuses on ensuring that essential health services are accessible to the community.
- Making sure that essential community oriented health services are available and meets the needs of the population

B. SCOPE OF PUBLIC HEALTH

1. Activities that must be Conducted on a Community Basis

- Supervision of community food, water, and milk supplies as well as medications, household products, toys, and recreational activities
- Insect, rodent, and other vector control
- Environmental pollution control including atmospheric, soil, and aquatic pollution, prevention of radiation hazards, and noise abatement

2. Activities Designed for Prevention of Illness, Disability, or Premature Death Caused By:

- Communicable disease including parasitic infections
- Dietary deficiencies or excess
- Behavioral disorders including alcoholism, drug habituation, narcotic addiction, certain aspects of delinquency, and suicide.
- Mental illness including mental retardation

- Allergic manifestations and their community sources
- Neoplastic Diseases
 - Neoplasm: unusual growth of cells aka tumor
- Acute and Chronic non-communicable respiratory diseases
- Metabolic Diseases
- Certain Hereditary or Genetic Conditions
- Occupational Diseases
 - Work/job diseases that are acquired through the job/work
- Home, vehicular, and industrial accidents
- Dental disorders including dental caries and periodontal disease
- Certain risks of maternity, growth, and development

3. Activities Related to Comprehensive Health Care

- Promotion of development, availability, and quality of health personnel, facilities, and services in the broadest sense
- Operation of programs for early detection of disease
- Promotion and sometimes operation of emergency medical service systems
- Promotion and sometimes operation of treatment centers
- Facilitation of and participation of continuing education

4. Activities Concerned with Collection, Preservation, Analysis, and Use of Vital Records

- Public health ensures the collection, preservation, analysis, and use of vital records, such as birth and death certificates, to monitor population health trends.

5. Public Education and Motivation in Personal and Community Health

- Educating and motivating individuals and communities about personal and public health fosters healthier lifestyles and environments.

6. Comprehensive Health Planning and Evolution

- Public health leads comprehensive health planning and evaluation, ensuring resources are allocated effectively to meet population needs.

7. Research: Scientific, Technical, and Administrative

- Public health supports scientific, technical, and administrative research, driving innovation and evidence-based practices to address emerging health challenges.

C. LEVELS OF HEALTHCARE

1. Health Promotion

- Focuses on activities that aim to improve or maintain the overall health status of individuals and communities. These activities target various aspects of daily life to support physical, mental, and emotional well-being.
- Examples include:
 - **Ensuring adequate rest for toddlers** to support healthy growth and development.
 - **Designing personality development programs for adolescents** to build confidence and resilience.

- **Retention of natural teeth** through proper oral hygiene and regular dental care.
- **Encouraging physical fitness and exercise** for individuals of all ages to promote long-term health.
- At the community level, health promotion may include services like:
 - **Family planning** to support reproductive health and well-being.
 - **Basic nutrition education** to encourage healthy eating habits and prevent deficiencies.

2. Disease Prevention

- Focuses on specific measures aimed at avoiding illness or disability. These efforts are categorized into two key areas:
- **Clinical Prevention:**
 - Involves medical interventions such as **immunizations** to prevent infectious diseases and **screening programs** to detect diseases early.
 - Also includes the **diagnosis and treatment of risk factors**, such as managing high blood pressure to prevent heart disease.
- **Behavioral Intervention:**
 - Focuses on **encouraging lifestyle changes** to reduce disease risk. Examples include promoting healthy eating, regular exercise, and smoking cessation.
- Preventive health efforts **address critical areas** such as:
 - Immunizations to protect against diseases like measles and polio.
 - Family planning to improve reproductive health.
 - Hypertension control to prevent complications like strokes and heart attacks.

- Treatment of sexually transmitted diseases (STDs) to reduce their spread

- At the community level, preventive measures include initiatives like **fluoridation of the water supply**, which helps reduce dental cavities across populations.
- By **focusing on prevention**, this level of healthcare reduces the burden of disease, improves quality of life, and promotes long-term health outcomes.

3. Diagnosis and Treatment

- Focuses on **restoring health** by **emphasizing early diagnosis and prompt treatment of health issues**.
- Early intervention can prevent complications and improve outcomes.
- Examples include:
 - **Individual Level:** Counseling adolescents for conditions such as sexually transmitted diseases (STDs).
 - **Family Level:** Providing counseling for marital problems to address mental and emotional well-being.

4. Rehabilitation

- Focuses on **limiting the impact of health problems, reducing disability, and preventing recurrences**.
- It helps individuals regain function and improve their quality of life.
- Example: Physical therapy for a post-stroke patient
 - aids recovery and prevents further complications.
 - Rehabilitation services are essential for helping individuals return to daily activities and achieve their highest level of independence.

Intent of the 1st and 2nd Levels

- Promote health and prevent disease

Intent of the 3rd and 4th Levels

- To prevent serious consequences arising from health problems

D. FOCUS ON COMMUNITY HEALTH PRACTICE

- These three levels highlight the comprehensive nature of community health practice, working proactively and reactively to improve the health and well-being of individuals and communities.

Primary Level Prevention

- Measures designed to **promote general optimum health** or the specific protection of man against disease agents
- These are actions taken to **prevent the occurrence of health problems**
- Includes **reducing the risk factors and preventing environmental exposures**
- e.g. eating nutritious food, routine immunizations, etc.

Secondary Level Prevention

- Focuses on the **early identification and treatment of existing health problems and after the occurrence** of health problems
- Encompasses activities designed to identify and manage conditions promptly, reducing their progression or complications.
- Helps minimize impacts of diseases through **timely interventions**
- e.g. screening of glaucoma, diagnosis and treatment of glaucoma

Tertiary Level Prevention

- Aimed at **returning the patient to the highest level of functioning** possible following treatment of a health problem
- corresponds to rehabilitative care, focusing on recovery and preventing further complications.
- Ensures patients can recover and regain independence while maintaining their health.
- e.g. placing the client on maintenance diet after the loss of weight due to illness

III. CONCEPTS OF COMMUNITY

Edward & Jones (1976)

- "A group of people who reside in a specific locality and exercise some degree of local autonomy in organizing their social life in such a way that they can base and satisfy the full range of their daily needs."

World Health Organization (WHO)

- "A social group determined by geographic boundaries and / or common values and interests; its members know and interact with one another; it functions within a particular social structure and exhibits and creates norms, values, and social institutions."
- "It is a collection of people who share some important features of their lives." (e.g. LGBT community, Filipino expat community)
- **Common in these definitions:**
 - Network of interpersonal relationships that provide friendship and support to members
 - Residence in common locality

- Solidarity, common sentiments and activities

A. COMMON ATTRIBUTES

- Critical attributes that define a group of people as a community

Group Orientation

- Through group membership, an individual gains access to skills, services, necessities, and amenities of life that one cannot provide on one's own
- Leads to **group orientation**, where the group's goals take over the individual goals

Bonds between Individuals

- In many forms such as lifestyle, shared ethnicity, culture, living in a specific geographic location, similar interests, goals, or occupations.
 - The type of bond between members determines the type of community.

B. CLASSIFICATIONS OF COMMUNITY

Urban

- **High density community**
- Socially heterogeneous population
- Complex structure, non-agricultural occupation, such as business and industry
- Characterized by complex interpersonal relations

Rural

- Usually **small**

- Occupation of population is usually farming, fishing, and food gathering
- Characterized by **primary group relations**; well knit and having a high degree of group feeling

Suburban

- An outlying part of a city or town
- A smaller place adjacent to or sometimes within community distance of a city
- Characterized by the **blending of urban and rural**

Other Types of Communities

- **Communities with Territorial Bonds**
 - Have specifically **defined territories and boundaries** that may be spatial (space), temporal, or both
 - This reflects the **“where and when” dimension** of the definition of community
 - E.g. Velez College graduates from 1990 to 2000 form an association
- **Communities with Relational Bonds**
 - Includes groups in which the **bonds between individuals is a common relationship** rather than specific boundaries
 - E.g. Healthcare workers
- None of the communities discussed are exclusive, any group may represent more than one type of community

C. COMPONENTS OF COMMUNITY

The Core

- Represents the **people that make up the community**

- Included are the demographics of the population as well as the values, beliefs, and history of the people
- A strong core with shared values can lead to a more unified and resilient community.

Eight Subsystems

- **Housing**
 - Shelter, lodging, and dwellings provided for number of people or for a community
 - Adequacy and availability of the housing facilities to the whole population
 - Housing laws or regulations governing the people
- **Education**
 - Laws, regulations, facilities, and activities affecting education
 - Ratio of educators to learners
 - Distribution of education facilities
 - Recipients of education
 - Informal education facilities and activities existing in the community
- **Fire and Safety**
 - Fire protection facilities and fire prevention activities and their distribution in the community
 - Police protection
- **Politics and Government**
 - Political structures present in the community
 - Decision making process / pattern of leadership style observed
 - E.g. democratic, republic, decentralized
- **Health**
 - Health facilities and activities in the community
 - Distribution of health facilities
 - Utilization of health services

Communication Systems

- Types of communication existing (e.g. telephones, mail, telegrams, internet, etc.)
- **Forms of communication:**
 - Verbal
 - Written
 - Non Verbal
- **Formal** and **Informal communication**
- **Economics**
 - General occupation of the population
 - Type of economic activities, such as production, distribution, marketing, and budgeting of goods
 - Income
- **Recreation**
 - Recreational activities or facilities present
 - Consumers of these recreations and their appropriateness

D. COMMUNITY HEALTH

- The attainment of the greatest possible **biological, psychological, and social well being of the community** as an entity and its individual members and is not just the sum of the health status of all community members
- Focuses on the **health status of a group** of people as collective entities rather than as individuals

IV. QUALITY OF HEALTH: COMMUNITY ACTION & PUBLIC INNOVATION

“Participation is essential to sustain health promotion action”

A. HEALTH PROMOTION IN PUBLIC HEALTH

Health Promotion

- The **process of enabling people to increase control over their health and improve it.**
- **Ottawa:** It represents a comprehensive, social, & political process; it not only embraces actions directed at strengthening the skills & capabilities of individuals, but is also action directed towards changing social, environmental, & economic conditions so as to alleviate public & individual health.
- **Focused on behavioral/lifestyle changes** such as smoking cessation, diet, and exercise

B. COMMUNITY ORGANIZING

Minkler and Wallerstein (1997)

- The process by which community groups are helped to identify common problems or goals, mobilize resources, and in other ways develop and implement strategies for reaching the goals they collectively have set

General Steps in Community Organizing

- **Himmelman (1992)**
 - "Communities should play a lead role in order to achieve real empowerment and not just community betterment."
- **5 Steps in Community Organizing**
 - Problem identification
 - Interface with Community
 - People Organization
 - Community Profile and Assessment
 - Goal Setting and Formulation of Strategies

- This **process of following the 5 steps** in Community Organizing is necessary in:
 - Maintaining Community Health
 - Understanding Community
 - Provide Relevant Data
- Gathering and analyzing these types of health data are part of the epidemiological study that can be conducted in community

C. CENTRALIZED SPECIALTY CARE

Current Model of Centralized Specialty Care

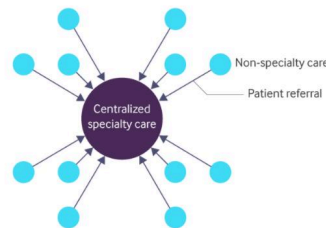


Figure 2. Current Model of Centralized Specialty Care

Figure 2 Discussion

- This model **centralizes resources and expertise in one location**, making it suitable for complex, rare conditions like pancreatic cancer surgery.
- Current centralizing models require patients to travel to a central hospital to obtain specialty services.
- These models are most appropriate for rare, complex services (e.g., pancreatic cancer surgery).

D. DECENTRALIZED HEALTH CARE SERVICES

A NEW ECOSYSTEM OF DISRUPTIVE BUSINESS MODELS MUST ARISE



Figure 3. Decentralized Health Care Services

Figure 3 Discussion

- The point of decentralized healthcare services is that instead of the community coming to a centralized healthcare facility, it is **the healthcare services that branch out into the community** instead

E. EFFECTS OF HEALTH TO THE COMMUNITY

- A healthy community produces a healthy citizenry
- A healthy community is an economically developed community
- The quality and quantity of human resources depend on the level of health of the people
- Less illness, disease, and disability imply a higher quality of life

V. REFERENCES

- Stanhope, M., & Lancaster, J. (2013). Foundations of Nursing in the Community: Community-Oriented Practice (4th ed.). Mosby.
- Howard, G. (2002). Healthy Villages. World Health Organization